

Definition of stigma:

"Stigma exists when elements of labeling, stereotyping, separation, status loss, and discrimination occur together..." (Link and Phelan, 2001).

Definitions of stigma domains:

- **Public stigma** consists of negative and misinformed stereotyped beliefs, prejudicial emotional responses, discriminatory behaviours.
- **Structural stigma** refers to systems, policies, laws that disempower, discriminate, and limit opportunities for people with lived or living experiences of mental illness.
- **Associative stigma** refers to stigma that is extended to family, carers, supporters, health workers, or others around people with lived or living experiences of mental illness.
- **Self-stigma** refers to internalised negative stereotypes in people with lived or living experiences of mental illness.

Task:

You are a psychology expert annotating stigma in media text. For each text that mentions a mental health condition, annotate:

1. Whether stigma is present and your confidence in your judgement.
2. Stigma mode: expressed/enacted and/or perceived/experienced.
3. Stigma domain and component:
 - a. Public
 - i. Cognitive (Stereotypes)
 - ii. Affective (Prejudicial emotions)
 - iii. Behavioral (Discrimination)
 - b. Structural
 - c. Associative
 - d. Self

Operational Rule for Stigma for task 1: YES / NO**Annotate "YES – Stigma present"**

Only if the text contains **at least one** of the following, **directed at a person or persons with a mental health condition** (or those associated with them):

- **Negative trait attribution** (e.g., dangerous, weak, incompetent, unpredictable)
- **Negative emotional reaction** (e.g., fear, disgust, contempt, ridicule)

- **Negative treatment or discrimination** (e.g., calls to isolate, punish, or deny rights/services, including discriminatory laws, rules, or institutional practices)
- **Derogatory or demeaning language** (e.g., slurs, insults)
- **Intrinsically stigmatizing usages of the MH condition** (e.g., metaphorical uses of the term in different contexts).

Do **not** annotate **YES** if:

- The negative attribution, reaction or treatment is not based on or related to the person's mental health condition (e.g., anger is expressed towards a person with depression because they committed a crime that is unrelated to their depression)
- The negative attribution, reaction or treatment is mentioned in text rather than endorsed or expressed (e.g., a journalist writes *about* negative attitudes towards schizophrenia without manifesting it herself; a person criticizing or commenting on stigma). **Note:** text that simply describes an incident of stigma perpetrated by someone else should be annotated YES: the key issue is whether the text displays stigma or comments/reflects on it.
- The negative attribution, reaction or treatment is one that is grounded in the situation and does not indicate a generalized social/cognitive processes here.

The hierarchy of stigma expressions (levels, types, and labels) are illustrated below:

Level 2: Stigma mode

1. Expressed/ Enacted
2. Perceived/ Experienced

Operational Rules for Stigma Mode

Stigma mode captures whether stigma is **Expressed/Enacted** by a stigmatizing source, **Perceived/Experienced** by a person with lived or living experience of mental illness, or **both**.

- Annotate **Expressed/Enacted stigma** when the text expresses stigma directly or describes stigma being enacted by a stigmatiser. This includes cases where a person, institution, policy, system, or media text attributes negative traits to people with mental health conditions, expresses negative emotional reactions toward them, or describes discriminatory treatment directed at them because of their condition.

- Annotate **Perceived/Experienced stigma** when the text describes stigma from the perspective of a person with lived or living experience of mental illness, including cases where that person reports being stereotyped, devalued, discriminated against, feared, excluded, mocked, or otherwise treated negatively because of their condition. This applies even when the text itself is reporting or reflecting on stigma rather than endorsing it.
- **Rule 1: First-person accounts.** If a person who identifies as having a mental health condition describes being stereotyped, devalued, discriminated against, feared, excluded, mocked, or otherwise treated negatively because of their condition, annotate this as **Perceived/Experienced stigma**.
- **Rule 2: Self-stigma.** When self-stigma is annotated, also annotate the mode as **Both: Expressed/Enacted and Perceived/Experienced stigma**. This is because self-stigma involves a person with lived or living experience internalising stigma as personally experienced, while also reproducing or expressing a negative stereotype about themselves as a person with mental illness.

Level 3: Stigma domain

1. Public
2. Structural
3. Associative
4. Self

Level 4: Stigma domain sub-factor (Public stigma only)

1. Cognitive (Stereotypes)
2. Affective (Prejudicial emotions)
3. Behavioral (Discrimination)

Level 5: Public Stigma Domain Sub-factor Components

Public: Cognitive

1. Dangerousness
2. Personal responsibility and blame
3. Uncontrollable
4. Controllable
5. Moral or character flaw
6. Pity/ condescension
7. Trivialisation or minimisation
8. Immutability

9. Segregation endorsement
10. Coercion
11. Contamination
12. Incompetence

Public: Affective

1. Fear
2. Anger, irritability or frustration
3. Disgust
4. Pity
5. Ambivalence

Public: Behavioural

1. Social distancing

The hierarchy of stigma expressions are elaborated below (with definitions and examples):

Level 2: Stigma mode

1. Expressed/ Enacted: Expression of stigma or description of its enactment by stigmatiser. (e.g., *"Police restrained the man using force after noting he had a history of psychiatric admissions."*)

2. Perceived/ Experienced: Stigma is perceived or its expression experienced by PWLE. (e.g., *"I was treated like I was dangerous", said Mark, who lives with bipolar disorder."*)

Level 3: Stigma domain

1. Public: Negative and misinformed stereotyped beliefs, prejudicial emotional responses, discriminatory behaviours. (e.g., *"Dr White said that, "Once people develop mental illness, they will never recover completely"."*)

2. Structural: Systems, policies, laws that disempower, discriminate, and limit opportunities for PWLE. (e.g., *"The report found that mental health patients waited twice as long for emergency care."*)

3. Associative: Stigma is extended to family, carers, supporters, health workers, or others around PWLE. (e.g., *"The ARAFMI representative said that mental illness is often concealed by families for fear of judgment."*)

4. Self: Internalisation of negative public stereotypes (e.g., *James, 34, said, "For a long time, I thought my mental illness made me a burden to everyone around me"*).

- **Reminder:** When annotating **Self-stigma**, also annotate the stigma mode as **Both (Expressed/Enacted and Perceived/Experienced)**.

Level 4: Stigma domain sub-factor (Public stigma only)

1. Cognitive (Stereotypes): Stereotypes are socially shared and generalised negative beliefs about mental illness and people living with it. They represent the cognitive component of stigma. (e.g., *His neighbour said that, "he could snap out of it if he just tried".*)

2. Affective (Prejudicial emotions): Prejudicial affective responses are the emotional reactions that arise from stereotypes about mental illness and people living with it. They represent the affective component of stigma. (e.g., *"Many feel uneasy around people who are mentally ill"*)

3. Behavioral (Discrimination): Discriminatory behaviours are the actions that result from stereotypes and prejudicial emotions about mental illness and people living with it. They constitute the behavioural component of stigma. (e.g., *"The employer admitted he wouldn't hire someone with a history of mental illness."*)

Level 5: Public Stigma Domain Sub-factor Components

Public: Cognitive

1. Dangerousness: A belief that someone living with mental illness is dangerous. (e.g., *"Harry poses a threat to others"*)

2. Personal responsibility and blame: A belief that someone living with mental illness is responsible and/or to blame for their experience. (e.g., *"It is Harry's fault that he is in his present condition."*)

3. Uncontrollable: A belief that someone is not in control of their experience of mental illness and the behaviour that may arise from it.. (e.g., *"Harry is unpredictable."*)

4. Controllable: A belief that someone is in control of their experience of mental illness and the behaviour that may arise from it.. (e.g., *"People with depression could snap out of it if they wanted"*)

5. Moral or character flaw: A belief that a person's mental illness is a reflection of a moral or character flaw. (e.g., *"Depression is a sign of personal weakness"*)

6. Pity/ condescension: A belief that someone living with mental illness is inferior or less capable. (e.g., *"I feel sorry for Harry."*)

7. Trivialisation or minimisation: A belief that someone's mental illness is not a real, legitimate, sufficiently severe or impactful condition. (e.g., *"Depression is not a real mental illness"*)

8. Immutability: A belief that a person cannot recover from their experience of mental illness. (e.g., *"People with mental illness cannot live a good, rewarding life."*)

9. Segregation endorsement: A belief that people with mental illness should be physically or socially separated from the community. (i.e., *"Harry should be kept away from others."*)

10. Coercion: A belief that people with mental illness must be controlled or forced into treatment because they lack insight or self-control. (e.g., *"I would force Harry to take his medication."*)

11. Contamination: A belief that mental illness is contagious or morally polluting. Association with someone with mental illness may damage one's status or well-being (e.g., *It is best to avoid people with depression so you don't become depressed yourself.*)

12. Incompetence: A belief that people with mental illness are incapable or unreliable in daily or occupational functioning. (e.g., *"People with a mental illness should not be given any responsibilities."*)

Public: Affective

1. Fear: An emotional response of fear in response to people living with mental illness. (e.g., *I feel afraid of Harry.*)

2. Anger, irritability or frustration: An emotional response of anger, irritability, or frustration in response to people living with mental illness. (i.e., *I feel irritated by Harry*)

3. Disgust: An emotional response of revulsion or moral contamination toward people with mental illness. (e.g., *I feel sickened by them.*)

4. Pity: A compassionate but patronising emotional response towards people with mental illness. (e.g., *I feel pity for Harry.*)

5. Ambivalence: A mixed or conflicted emotional response involving simultaneous positive and negative feelings toward people with mental illness. (e.g., *"I feel both compassion and discomfort toward them."*)

Public: Behavioural

1. Social distancing: Behaviour discrimination in terms of desire for or actual limitation of the degree to which someone with mental illness is included in one's life across numerous contexts. (e.g., *"I stay away from Harry."*)